Pharmaceutical Procurement Diagnostic

Uncovering a systemic fulfilment failure hidden behind a healthy headline metric

Anonymised case study — national health authority, MENA region

A 73% Supply Rate Concealed a Critical "All-or-Nothing" Fulfilment Failure

The Situation

The headline pharmaceutical supply rate stood at 72.8% — an apparently stable figure. Deeper analysis revealed a systemic fulfilment problem: nearly half of all requested items were never delivered at all.

The Impact

An unfulfilled demand gap equal to 27% of total procurement value, with 44% of orders receiving nothing at all — alongside recurring stockouts of essential medicines.

The Recommendation

A three-pillar strategy: strategic supplier management, proactive inventory planning, and process optimisation.

The Headline Metric Suggested Stability: An Overall Supply Rate of 72.8%

72.8%

Total Demand
257.0M local currency
53M units

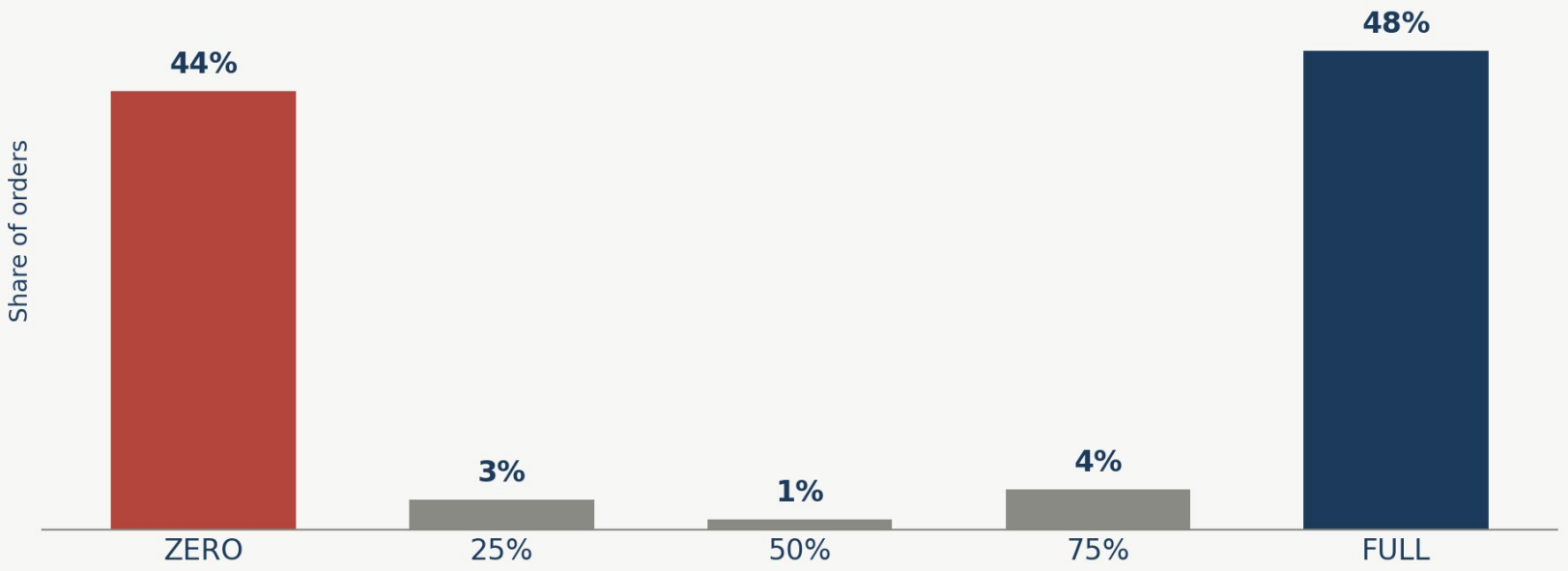
Total Received
194.0M local currency
39M units

Supply Gap
68.6M local currency
14M units

The Core Problem: Procurement Follows a High-Risk, "All-or-Nothing" Pattern

The overall 73% supply rate is an average of two extremes. Most orders are either fulfilled completely or not at all, with very few partial fulfilments. This points to a systemic issue, not random under-supply.

Distribution of Order Fulfilment



◂ A large portion of orders go completely unfilled

Another large portion are perfectly fulfilled ▸

This Fulfilment Failure Translates to Critical Stockouts of Essential Medicines

Item	Demand (units)	Fulfilment
Entecavir 0.5mg	3,965	0%
Ondansetron 4mg/2ml	3,778	0%
Heparin Sodium 5000 IU	2,450	0%
Vancomycin 500mg	1,200	0%
Propofol 1% 20ml	950	0%

Total value of zero-supply items amounts to 35.3M local currency across 1,607 orders.

The Approach

A six-week diagnostic run on existing procurement data — no new systems, no long onboarding.

Deliverables: fulfilment analysis by supplier and item · supplier segmentation matrix · prioritised recommendations · executive presentation.

Built by an operator, not a consultant.

Pharmacist by training, MBA. I have owned and run a medical consumables and pharmaceutical distribution company since 2018 — and I analyse these decisions with my own money before analysing anyone else's.

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